



ARIZONA HOUSE OF REPRESENTATIVES FLOOR AMENDMENT EXPLANATION

57th Legislature, 2nd Regular Session
Majority Research Staff

SB 1242: mental health; hearings; audiovisual technology

BLISS FLOOR AMENDMENT

1. Modifies the definition of a *mental health treatment agency*.
2. Makes further technical changes and revisions relating to behavioral health facilities.
3. Contains a retroactivity clause.

ADDITIONAL COW
BLISS FLOOR AMENDMENT
HOUSE OF REPRESENTATIVES AMENDMENTS TO S.B. 1242
(Reference to Senate engrossed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.
[Green underlining in brackets] indicates text added to new session law or text restoring existing law.
[GREEN STRIKEOUT IN BRACKETS] indicates new text removed from statute or previously enacted session law.
[Green strikeout in brackets] indicates text removed from existing statute, previously enacted session law or new session law.
<<Green carets>> indicate a section added to the bill.
<<Green strikeout in carets>> indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 <<Section 1. Section 36-501, Arizona Revised Statutes, is amended
3 to read:

4 36-501. Definitions

5 In this chapter, unless the context otherwise requires:

6 1. "Administration" means the Arizona health care cost containment
7 system administration.

8 2. "Admitting officer" means a psychiatrist or other physician or
9 psychiatric and mental health nurse practitioner with experience in
10 performing psychiatric examinations who has been designated as an
11 admitting officer of the evaluation agency by the person in charge of the
12 evaluation agency.

13 3. "Authorized transporter" means a transportation entity that is
14 contracted with a city, town or county to provide services pursuant to
15 this chapter and that is either:

16 (a) An ambulance service that holds a valid certificate of
17 necessity.

18 (b) A transportation provider authorized by this state to provide
19 safe behavioral health transportation for individuals requiring
20 transportation pursuant to this chapter.

21 4. "Chief medical officer" means the chief medical officer under
22 the supervision of the superintendent of the state hospital.

23 5. "Contraindicated" means that access is reasonably likely to
24 endanger the life or physical safety of the patient or another person.

25 6. "Court" means the superior court in the county in this state in
26 which the patient resides or was found before screening or emergency
27 admission under this title.

28 7. "Criminal history" means police reports, lists of prior arrests
29 and convictions, criminal case pleadings and court orders, including a
30 determination that the person has been found incompetent to stand trial
31 pursuant to section 13-4510.

1 8. "Danger to others" means that the judgment of a person who has a
2 mental disorder is so impaired that the person is unable to understand the
3 person's need for treatment and as a result of the person's mental
4 disorder the person's continued behavior can reasonably be expected, on
5 the basis of competent medical opinion, to result in serious physical
6 harm.

7 9. "Danger to self":

8 (a) Means behavior that, as a result of a mental disorder:

9 (i) Constitutes a danger of inflicting serious physical harm on
10 oneself, including attempted suicide or the serious threat thereof, if the
11 threat is such that, when considered in the light of its context and in
12 light of the individual's previous acts, it is substantially supportive of
13 an expectation that the threat will be carried out.

14 (ii) Without hospitalization will result in serious physical harm
15 or serious illness to the person.

16 (b) Does not include behavior that establishes only the condition
17 of having a grave disability.

18 10. "Department" means the department of health services.

19 11. "Detention" means the taking into custody of a patient or
20 proposed patient.

21 12. "Director" means the director of the administration.

22 13. "Evaluation" means:

23 (a) A professional multidisciplinary analysis that may include
24 firsthand observations or remote observations by interactive audiovisual
25 media and that is based on data describing the person's identity,
26 biography and medical, psychological and social conditions carried out by
27 a group of persons consisting of at least the following:

28 (i) Two licensed physicians who are qualified psychiatrists, if
29 possible, or at least experienced in psychiatric matters, who shall
30 examine and report their findings independently. The person against whom
31 a petition has been filed shall be notified that the person may select one
32 of the physicians. A psychiatric resident in a training program approved
33 by the American medical association or by the American osteopathic
34 association may examine the person in place of one of the psychiatrists if
35 the resident is supervised in the examination and preparation of the
36 affidavit and testimony in court by a qualified psychiatrist appointed to
37 assist in the resident's training, and if the supervising psychiatrist is
38 available for discussion with the attorneys for all parties and for court
39 appearance and testimony if requested by the court or any of the
40 attorneys.

41 (ii) Two other individuals, one of whom, if available, is a
42 psychologist and in any event a social worker familiar with mental health
43 and human services that may be available placement alternatives
44 appropriate for treatment. An evaluation may be conducted on an inpatient
45 basis, an outpatient basis or a combination of both, and every reasonable

1 attempt shall be made to conduct the evaluation in any language preferred
2 by the person.

3 (b) A physical examination that is consistent with the existing
4 standards of care and that is performed by one of the evaluating
5 physicians or by or under the supervision of a physician who is licensed
6 pursuant to title 32, chapter 13 or 17 or a registered nurse practitioner
7 who is licensed pursuant to title 32, chapter 15 if the results of that
8 examination are reviewed or augmented by one of the evaluating physicians.

9 14. "Evaluation agency" means either of the following:

10 (a) A health care agency that is licensed by the department and
11 that has been approved pursuant to this title to provide the services
12 required of that agency by this chapter.

13 (b) A facility that is exempt from licensure pursuant to section
14 36-402, that possesses an accreditation from either a national commission
15 on correctional health care or an American correctional association and
16 that has been approved pursuant to this title to provide the services
17 required of that facility by this chapter.

18 15. "Family member" means a spouse, parent, adult child, adult
19 sibling or other blood relative of a person undergoing treatment or
20 evaluation pursuant to this chapter.

21 16. "Grave disability" means a condition evidenced by behavior in
22 which a person, as a result of a mental disorder, is likely to come to
23 serious physical harm or serious illness because the person is unable to
24 provide for the person's own basic physical needs.

25 17. "Health care decision maker" has the same meaning prescribed in
26 section 12-2801.

27 18. "Health care entity" means a health care provider, the
28 department, the administration or a regional behavioral health authority
29 that is under contract with the administration.

30 19. "Health care provider" means a health care institution as
31 defined in section 36-401 that is licensed as a behavioral health provider
32 pursuant to department rules or a mental health provider.

33 20. "Independent evaluator" means a licensed physician, psychiatric
34 and mental health nurse practitioner or psychologist who is selected by
35 the person to be evaluated or by the person's attorney.

36 21. "Informed consent" means a voluntary decision following **THE**
37 presentation of all facts necessary to form the basis of an intelligent
38 consent by the patient or guardian with no minimizing of known dangers of
39 any procedures.

40 22. "Least restrictive treatment alternative" means the treatment
41 plan and setting that infringe in the least possible degree with the
42 patient's right to liberty and that are consistent with providing needed
43 treatment in a safe and humane manner.

44 23. "Licensed physician" means any medical doctor or doctor of
45 osteopathy who is either:

46 (a) Licensed in this state.

1 (b) A full-time hospital physician licensed in another state and
2 serving on the staff of a hospital operated or licensed by the United
3 States government.

4 24. "Medical director of an evaluation agency" means a
5 psychiatrist, or other licensed physician experienced in psychiatric
6 matters, who is designated in writing by the governing body of the agency
7 as the person in charge of the medical services of the agency for the
8 purposes of this chapter and may include the chief medical officer of the
9 state hospital.

10 25. "Medical director of a mental health treatment agency" means a
11 psychiatrist, or other licensed physician experienced in psychiatric
12 matters, who is designated in writing by the governing body of the agency
13 as the person in charge of the medical services of the agency for the
14 purposes of this chapter and includes the chief medical officer of the
15 state hospital.

16 26. "Mental disorder" means a substantial disorder of the person's
17 emotional processes, thought, cognition or memory. Mental disorder is
18 distinguished from:

19 (a) Conditions that are primarily those of drug abuse, alcoholism
20 or intellectual disability, unless, in addition to one or more of these
21 conditions, the person has a mental disorder.

22 (b) The declining mental abilities that directly accompany
23 impending death.

24 (c) Character and personality disorders characterized by lifelong
25 and deeply ingrained antisocial behavior patterns, including sexual
26 behaviors that are abnormal and prohibited by statute unless the behavior
27 results from a mental disorder.

28 27. "Mental health provider" means any physician or provider of
29 mental health or behavioral health services who is involved in evaluating,
30 caring for, treating or rehabilitating a patient.

31 28. "Mental health treatment agency"[;]

32 [(a)] Means any of the following:

33 [(a)] [(i)] The state hospital.

34 [(a)] [(ii)] A health care agency that is licensed by the
35 department and that provides the services that are required of the agency
36 by this chapter.

37 [(a)] [(iii)] A facility that is exempt from licensure pursuant to
38 section 36-402, that possesses an accreditation from either a national
39 commission on correctional health care or an American correctional
40 association and that provides the services that are required of the
41 facility by this chapter.

42 [(b) DOES NOT INCLUDE A SECURE BEHAVIORAL HEALTH RESIDENTIAL
43 FACILITY THAT PROVIDES SERVICES ONLY TO PERSONS PLACED IN THE FACILITY
44 PURSUANT TO A COURT ORDER ISSUED PURSUANT TO SECTION 36-550.09.]

1 29. "Outpatient treatment" or "combined inpatient and outpatient
2 treatment" means any treatment program [THAT DOES] not ~~[requiring]~~
3 [REQUIRE] continuous inpatient hospitalization.

4 30. "Outpatient treatment plan" means a treatment plan that does
5 not require continuous inpatient hospitalization.

6 31. "Patient" means any person who is undergoing examination,
7 evaluation or behavioral or mental health treatment under this chapter.

8 32. "Peace officers" means sheriffs of counties, constables,
9 marshals and policemen of cities and towns.

10 33. "Persistent or acute disability" means a severe mental disorder
11 that meets all the following criteria:

12 (a) Significantly impairs judgment, reason, behavior or capacity to
13 recognize reality.

14 (b) If not treated, has a substantial probability of causing the
15 person to suffer or continue to suffer severe and abnormal mental,
16 emotional or physical harm.

17 (c) Substantially impairs the person's capacity to make an informed
18 decision regarding treatment, and this impairment causes the person to be
19 incapable of understanding and expressing an understanding of the
20 advantages and disadvantages of accepting treatment and understanding and
21 expressing an understanding of the alternatives to the particular
22 treatment offered after the advantages, disadvantages and alternatives are
23 explained to that person.

24 (d) Has a reasonable prospect of being treatable by outpatient,
25 inpatient or combined inpatient and outpatient treatment.

26 34. "Prepetition screening" means the review of each application
27 requesting court-ordered evaluation, including an investigation of facts
28 alleged in the application, an interview with each applicant and an
29 interview, if possible, with the proposed patient. The purpose of the
30 interview with the proposed patient is to assess the problem, explain the
31 application and, when indicated, attempt to persuade the proposed patient
32 to receive, on a voluntary basis, evaluation or other services.

33 35. "Prescribed form" means a form established by a court or the
34 rules of the administration in accordance with the laws of this state.

35 36. "Professional" means a physician who is licensed pursuant to
36 title 32, chapter 13 or 17, a psychologist who is licensed pursuant to
37 title 32, chapter 19.1 or a psychiatric and mental health nurse
38 practitioner who is certified pursuant to title 32, chapter 15.

39 37. "Proposed patient" means a person for whom an application for
40 evaluation has been made or a petition for court-ordered evaluation has
41 been filed.

42 38. "Prosecuting agency" means the county attorney, attorney
43 general or city attorney who applied or petitioned for an evaluation or
44 treatment pursuant to this chapter.

45 39. "Psychiatric and mental health nurse practitioner" means a
46 registered nurse practitioner as defined in section 32-1601 who has

1 completed an adult or family psychiatric and mental health nurse
2 practitioner program and who is certified as an adult or family
3 psychiatric and mental health nurse practitioner by the state board of
4 nursing.

5 40. "Psychiatrist" means a licensed physician who has completed
6 three years of graduate training in psychiatry in a program approved by
7 the American medical association or the American osteopathic association.

8 41. "Psychologist" means a person who is licensed under title 32,
9 chapter 19.1 and who is experienced in the practice of clinical
10 psychology.

11 42. "Records" means all communications that are recorded in any
12 form or medium and that relate to patient examination, evaluation or
13 behavioral or mental health treatment. Records include medical records
14 that are prepared by a health care provider or other providers. Records
15 do not include:

16 (a) Materials that are prepared in connection with utilization
17 review, peer review or quality assurance activities, including records
18 that a health care provider prepares pursuant to section 36-441, 36-445,
19 36-2402 or 36-2917.

20 (b) Recorded telephone and radio calls to and from a publicly
21 operated emergency dispatch office relating to requests for emergency
22 services or reports of suspected criminal activity.

23 43. "Regional behavioral health authority" has the same meaning
24 prescribed in section 36-3401.

25 44. "Screening agency" means a health care agency that is licensed
26 by the department and that provides those services required of the agency
27 by this chapter.

28 45. "Social worker" means a person who has completed two years of
29 graduate training in social work in a program approved by the council of
30 social work education and who has experience in mental health.

31 46. "State hospital" means the Arizona state hospital.

32 47. "Superintendent" means the superintendent of the state
33 hospital.

34 48. "Voluntary evaluation" means the ongoing collection and
35 analysis of a person's medical, psychological, psychiatric and social
36 conditions in order to initially determine if a health disorder exists and
37 if there is a need for behavioral health services and, on an ongoing
38 basis, to ensure that the person's service plan is designed to meet the
39 person's and the person's family's current needs and long-term goals.>>

40 Sec. 2. Section 36-505, Arizona Revised Statutes, is amended to
41 read:

42 36-505. Rights at hearing; virtual proceedings

43 A. At all hearings conducted pursuant to this chapter, persons
44 shall have the right to an analysis of their psychological condition by an
45 independent evaluator.

1 B. IN ANY PROCEEDING HELD PURSUANT TO THIS CHAPTER, THE COURT, ON
2 ITS OWN OR FOR GOOD CAUSE SHOWN BY A PARTY, MAY ORDER THE HEARING TO BE
3 CONDUCTED OR [MAY ALLOW] A WITNESS OR PARTY ~~[MAY BE ALLOWED]~~ TO APPEAR AND
4 GIVE TESTIMONY VIRTUALLY THROUGH THE USE OF TELEPHONE, VIDEOCONFERENCING
5 OR OTHER AUDIOVISUAL TECHNOLOGY IF THE COURT FINDS THAT THIS PROCEDURE
6 WILL NOT UNFAIRLY PREJUDICE A PARTY OR WITNESS. IN DETERMINING WHETHER A
7 PROCEEDING SHOULD BE CONDUCTED WHOLLY OR PARTIALLY WITH THE USE OF
8 TELEPHONE, VIDEOCONFERENCING OR OTHER AUDIOVISUAL TECHNOLOGY, THE COURT
9 SHALL CONSIDER THE INCONVENIENCE TO OR BURDEN ON A PARTY OR WITNESS TO
10 ATTEND THE PROCEEDING IN PERSON AND THE ABILITY OF ALL PARTIES TO BE HEARD
11 BY OTHER PARTIES IN ATTENDANCE, INCLUDING THE JUDICIAL OFFICER AND, IF
12 APPLICABLE, THE CERTIFIED REPORTER OR AN ELECTRONIC RECORDING SYSTEM. THE
13 SUPREME COURT MAY ADOPT RULES TO GOVERN THE PROCEDURES TO BE USED IN
14 CONDUCTING PROCEEDINGS VIRTUALLY PURSUANT TO THIS CHAPTER.

15 Sec. 3. Section 36-539, Arizona Revised Statutes, is amended to
16 read:

17 36-539. Conduct of hearing; record; transcript

18 A. The medical director of the evaluation agency shall issue
19 instructions to the physicians or the psychiatric and mental health nurse
20 practitioner of the evaluation agency who is treating the proposed patient
21 to take all reasonable precautions to ensure that at the time of the
22 hearing the proposed patient is not so under the influence of or does not
23 so suffer the effects of drugs, medication or other treatment as to be
24 hampered in preparing for or participating in the hearing. If the
25 proposed patient is being treated as an inpatient by the evaluation
26 agency, the court at the time of the hearing shall be presented a record
27 of all drugs, medication or other treatment that the person has received
28 during the seventy-two hours immediately before the hearing.

29 B. The patient and the patient's attorney shall be present at all
30 hearings, and the patient's attorney may subpoena and cross-examine
31 witnesses and present evidence. THE COURT, FOR GOOD CAUSE SHOWN, MAY
32 ALLOW THE PATIENT, THE PATIENT'S ATTORNEY OR ANY WITNESS TO APPEAR AND
33 PRESENT TESTIMONY IN OPEN COURT THROUGH THE USE OF TELEPHONE,
34 VIDEOCONFERENCING OR OTHER AUDIOVISUAL TECHNOLOGY. The patient may choose
35 to not attend the hearing or the patient's attorney may waive the
36 patient's presence. The evidence presented by the petitioner or the
37 patient shall include the testimony of two or more witnesses, regardless
38 of the witnesses' professional licensure, if any, who observed or were
39 acquainted with the patient at the time of the alleged mental disorder
40 before the submission of the current application for evaluation pursuant
41 to section 36-520 or, if after the submission of the current application,
42 who were not formal participants in the evaluation process. The testimony
43 of the witnesses shall be limited to observed facts and may not include
44 expert opinion or conclusions. The witness testimony may be satisfied by
45 a statement agreed on by the parties and testimony of the two physicians
46 or other health professionals who participated in the evaluation of the

1 patient pursuant to section 36-533, which may be satisfied by stipulating
2 to the admission of the affidavits as required pursuant to section 36-533,
3 subsection B. The evaluating physicians or other health professionals
4 shall testify as to their personal observations of the patient. They
5 shall also testify as to their opinions concerning whether the patient is,
6 as a result of mental disorder, a danger to self or to others or has a
7 persistent or acute disability or a grave disability and as to whether the
8 patient requires treatment. Such testimony shall state specifically the
9 nature and extent of the danger to self or to others, the persistent or
10 acute disability or the grave disability. If the patient has a grave
11 disability, the evaluating physicians or other health professionals shall
12 testify concerning the need for guardianship or conservatorship, or both,
13 and whether or not the need is for immediate appointment. Other persons
14 who have participated in the evaluation of the patient or, if further
15 treatment was requested by a mental health treatment agency, persons of
16 that agency who are directly involved in the care of the patient shall
17 testify at the request of the court or of the patient's attorney.
18 Witnesses shall testify as to placement alternatives appropriate and
19 available for the care and treatment of the patient. The clinical record
20 of the patient for the current admission shall be available and may be
21 presented in full or in part as evidence at the request of the court, the
22 county attorney or the patient's attorney.

23 C. If the patient, for medical or psychiatric reasons, is unable to
24 be present at the hearing and cannot appear by other reasonably feasible
25 means, the court shall require clear and convincing evidence that the
26 patient is unable to be present at the hearing and on such a finding may
27 proceed with the hearing in the patient's absence.

28 D. The requirements of subsection B of this section are in addition
29 to all rules of evidence and the Arizona rules of civil procedure, not
30 inconsistent with subsection B of this section.

31 E. A verbatim record of all proceedings under this section shall be
32 made by stenographic means by a court reporter if a written request for a
33 court reporter is made by any party to the proceedings at least
34 twenty-four hours in advance of such proceedings. If stenographic means
35 are not requested in the manner provided by this subsection, electronic
36 means shall be directed by the presiding judge. The stenographic notes or
37 electronic tape shall be retained as provided by statute.

38 F. A patient who has been ordered to undergo treatment may request
39 a certified transcript of the hearing. To obtain a copy, the patient
40 shall pay for a transcript or shall file an affidavit that the patient is
41 without means to pay for a transcript. If the affidavit is found true by
42 the court, the expense of the transcript is a charge on the county in
43 which the proceedings were held, or, if an intergovernmental agreement by
44 the counties has required evaluation in a county other than that of the
45 patient's residence, such expense may be charged to the county of the
46 patient's residence or in which the patient was found before evaluation.

1 <<Sec. 4. Section 36-540, Arizona Revised Statutes, is amended to
2 read:

3 36-540. Court options; immunity; rules

4 A. If the court finds by clear and convincing evidence that the
5 proposed patient, as a result of mental disorder, is a danger to self, is
6 a danger to others or has a persistent or acute disability or a grave
7 disability and is in need of treatment, and is either unwilling or unable
8 to accept voluntary treatment, the court shall order the patient to
9 undergo one of the following:

10 1. Treatment in a program of outpatient treatment.

11 2. Treatment in a program consisting of combined inpatient and
12 outpatient treatment.

13 3. Inpatient treatment in a mental health treatment agency, in a
14 hospital operated by or under contract with the United States department
15 of veterans affairs to provide treatment to eligible veterans pursuant to
16 article 9 of this chapter, in the state hospital or in a private hospital,
17 if the private hospital agrees, subject to the limitations of section
18 36-541.

19 B. The court shall consider all available and appropriate
20 alternatives for the treatment and care of the patient. The court shall
21 order the least restrictive treatment alternative available.

22 C. The court may order the proposed patient to undergo outpatient
23 [TREATMENT] or combined inpatient and outpatient treatment pursuant to
24 subsection A, paragraph 1 or 2 of this section if the court:

25 1. Determines that all of the following apply:

26 (a) The patient does not require continuous inpatient
27 hospitalization.

28 (b) The patient will be more appropriately treated in an outpatient
29 treatment program or in a combined inpatient and outpatient treatment
30 program.

31 (c) The patient will follow a prescribed outpatient treatment plan.

32 (d) The patient will not likely become dangerous or suffer more
33 serious physical harm or serious illness or further deterioration if the
34 patient follows a prescribed outpatient treatment plan.

35 2. Is presented with and approves a written treatment plan that
36 conforms with the requirements of section 36-540.01, subsection B. ~~[If
37 the court determines that the patient meets the requirements of section
38 36-550.09, the court may order the patient to be placed in a secure
39 behavioral health residential facility that is licensed by the department
40 pursuant to section 36-425.06.]~~ If the treatment plan presented to the
41 court pursuant to this subsection provides for supervision of the patient
42 under court order by a mental health [TREATMENT] agency that is other than
43 the mental health [TREATMENT] agency that petitioned or requested the
44 county attorney to petition the court for treatment pursuant to section
45 36-531, the treatment plan must be approved by the medical director of the

1 mental health [TREATMENT] agency that will supervise the treatment
2 pursuant to subsection E of this section.

3 D. An order to receive treatment pursuant to subsection A,
4 paragraph 1 or 2 of this section shall not exceed three hundred sixty-five
5 days. The period of inpatient treatment under a combined [INPATIENT AND
6 OUTPATIENT] treatment order pursuant to subsection A, paragraph 2 of this
7 section shall not exceed the maximum period allowed for an order for
8 inpatient treatment pursuant to subsection F of this section.

9 E. If the court enters an order for treatment pursuant to
10 subsection A, paragraph 1 or 2 of this section, all of the following
11 apply:

12 1. The court shall designate the medical director of the mental
13 health treatment agency that will supervise and administer the patient's
14 treatment program.

15 2. The medical director [OF THE MENTAL HEALTH TREATMENT AGENCY]
16 shall not use the services of any person, agency or organization to
17 supervise a patient's outpatient treatment program unless the person,
18 agency or organization has agreed to provide these services in the
19 individual patient's case and unless the department has determined that
20 the person, agency or organization is capable and competent to do so.

21 3. The person, agency or organization assigned to supervise an
22 outpatient treatment program or the outpatient portion of a combined
23 [INPATIENT AND OUTPATIENT] treatment program shall be notified at least
24 three days before a referral. The medical director [OF THE MENTAL HEALTH
25 TREATMENT AGENCY] making the referral and the person, agency or
26 organization assigned to supervise the treatment program shall share
27 relevant information about the patient to provide continuity of treatment.

28 4. The court may order the medical director [OF THE MENTAL HEALTH
29 TREATMENT AGENCY] to provide notice to the court of any noncompliance with
30 the terms of a treatment order.

31 [5. IF THE COURT DETERMINES THAT THE PATIENT MEETS THE REQUIREMENTS
32 OF SECTION 36-550.09 AND ORDERS THE PATIENT TO BE PLACED IN A SECURE
33 BEHAVIORAL HEALTH RESIDENTIAL FACILITY THAT IS LICENSED BY THE DEPARTMENT
34 PURSUANT TO SECTION 36-425.06, THE MENTAL HEALTH TREATMENT AGENCY THAT
35 WILL SUPERVISE THE TREATMENT PURSUANT TO THIS SUBSECTION AND THE SECURE
36 BEHAVIORAL HEALTH RESIDENTIAL FACILITY SHALL COORDINATE THE CARE OF THE
37 PATIENT PURSUANT TO RULES ADOPTED BY THE DEPARTMENT OR AS REQUIRED BY THE
38 ADMINISTRATION.]

39 [5.] [6.] During any period of outpatient treatment under
40 subsection A, paragraph 2 of this section, if the court, on its own
41 motion, on request of a guardian pursuant to paragraph [7] [8] of this
42 subsection or on motion by the medical director of the patient's
43 outpatient mental health treatment facility, determines that the patient
44 is not complying with the terms of the order or that the outpatient
45 treatment plan is no longer appropriate and the patient needs inpatient
46 treatment, the court, without a hearing and based on the court record, the

1 patient's medical record, the affidavits and recommendations of the
 2 medical director [OF THE MENTAL HEALTH TREATMENT AGENCY], and the advice
 3 of staff and physicians or the psychiatric and mental health nurse
 4 practitioner familiar with the treatment of the patient, may enter an
 5 order amending its original order. The amended order may alter the
 6 outpatient treatment plan or order the patient to inpatient treatment
 7 pursuant to subsection A, paragraph 3 of this section. The amended order
 8 shall not increase the total period of commitment originally ordered by
 9 the court or, when added to the period of inpatient treatment provided by
 10 the original order and any other amended orders, exceed the maximum period
 11 allowed for an order for inpatient treatment pursuant to subsection F of
 12 this section. If the patient refuses to comply with an amended order for
 13 inpatient treatment, the court, on its own motion or on the request of the
 14 medical director [OF THE MENTAL HEALTH TREATMENT AGENCY], may authorize
 15 and direct a peace officer to take the patient into protective custody and
 16 transport the patient to the [MENTAL HEALTH TREATMENT] agency for
 17 inpatient treatment. Any authorization, directive or order issued to a
 18 peace officer to take the patient into protective custody shall include
 19 the patient's criminal history and the name and telephone numbers of the
 20 patient's case manager, guardian, spouse, next of kin or significant
 21 other, as applicable. When reporting to or being returned to a [MENTAL
 22 HEALTH] treatment agency for inpatient treatment pursuant to an amended
 23 order, the patient shall be informed of the patient's right to judicial
 24 review and the patient's right to consult with counsel pursuant to section
 25 36-546.

26 ~~[6.]~~ [7.] During any period of outpatient treatment under
 27 subsection A, paragraph 2 of this section, if the medical director of the
 28 outpatient treatment facility in charge of the patient's care determines,
 29 in concert with the medical director of an inpatient mental health
 30 treatment facility who has agreed to accept the patient, that the patient
 31 is in need of immediate acute inpatient psychiatric care because of
 32 behavior that is dangerous to self or to others, the medical director of
 33 the outpatient treatment facility may order a peace officer to apprehend
 34 and transport the patient to the inpatient treatment facility pending a
 35 court determination on an amended order under paragraph ~~[5.]~~ [6] of this
 36 subsection. The patient may be detained and treated at the inpatient
 37 treatment facility for a period of not more than forty-eight hours,
 38 ~~[exclusive of]~~ [EXCLUDING] weekends and holidays, from the time that the
 39 patient is taken to the inpatient treatment facility. The medical
 40 director of the outpatient treatment facility shall file the motion for an
 41 amended court order requesting inpatient treatment not later than the next
 42 working day following the patient being taken to the inpatient treatment
 43 facility. Any period of detention within the inpatient treatment facility
 44 pending issuance of an amended order shall not increase the total period
 45 of commitment originally ordered by the court or, when added to the period
 46 of inpatient treatment provided by the original order and any other

1 amended orders, exceed the maximum period allowed for an order for
2 inpatient treatment pursuant to subsection F of this section. If a
3 patient is ordered to undergo inpatient treatment pursuant to an amended
4 order, the medical director of the outpatient treatment facility shall
5 inform the patient of the patient's right to judicial review and to
6 consult with an attorney pursuant to section 36-546.

7 ~~[7.]~~ [8.] If there is a court order for treatment and a
8 guardianship with additional mental health authority pursuant to section
9 14-5312.01 existing at the same time, the treatment and placement
10 decisions made by the [MENTAL HEALTH] treatment agency assigned by the
11 court to supervise and administer the patient's treatment program pursuant
12 to the court order for treatment are controlling unless the court orders
13 otherwise. During any period of outpatient treatment, the guardian of a
14 patient may file a report with the court that addresses whether the
15 patient is complying with the terms of the order, whether the outpatient
16 treatment plan is still appropriate and whether the patient needs
17 inpatient treatment. The report shall state in detail the facts on which
18 the guardian relies and may include other supporting documents. A copy of
19 the report and other supporting documents shall be given to the patient's
20 attorney and the outpatient [MENTAL HEALTH] treatment agency. After
21 reviewing the report and any supporting documents filed with the report,
22 if the court determines that there is reasonable cause to believe that the
23 patient is not complying with the terms of the order, that the outpatient
24 treatment plan is no longer appropriate or that the patient needs
25 inpatient treatment, the court must set a conference or a hearing or take
26 other action described in paragraph ~~[5-]~~ [6] of this subsection.

27 F. The maximum periods of inpatient treatment that the court may
28 order, subject to the limitations of section 36-541, subsection D, are as
29 follows:

- 30 1. Ninety days for a person found to be a danger to self.
- 31 2. One hundred eighty days for a person found to be a danger to
32 others.
- 33 3. One hundred eighty days for a person found to have a persistent
34 or acute disability.
- 35 4. Three hundred sixty-five days for a person found to have a grave
36 disability.

37 G. If, on finding that the patient meets the criteria for
38 court-ordered treatment pursuant to subsection A of this section, the
39 court also finds that there is reasonable cause to believe that the
40 patient is an incapacitated person as defined in section 14-5101 or is a
41 person in need of protection pursuant to section 14-5401 and that the
42 patient is or may be in need of guardianship or conservatorship, or both,
43 the court may order an investigation concerning the need for a guardian or
44 conservator, or both, and may appoint a suitable person or agency to
45 conduct the investigation. The appointee may include a court-appointed
46 guardian ad litem, an investigator appointed pursuant to section 14-5308

1 or the public fiduciary if there is no person willing and qualified to act
2 in that capacity. The court shall give notice of the appointment to the
3 appointee within three days after the appointment. The appointee shall
4 submit the report of the investigation to the court within twenty-one
5 days. The report shall include recommendations as to who should be
6 guardian or who should be conservator, or both, and a report of the
7 findings and reasons for the recommendation. If the investigation and
8 report so indicate, the court shall order the appropriate person to submit
9 a petition to become the guardian or conservator, or both, of the patient.

10 H. In any proceeding for court-ordered treatment in which the
11 petition alleges that the patient is in need of a guardian or conservator
12 and states the grounds for that allegation, the court may appoint an
13 emergency temporary guardian or conservator, or both, for a specific
14 purpose or purposes identified in its order and for a specific period of
15 time not to exceed thirty days if the court finds that all of the
16 following are true:

17 1. The patient meets the criteria for court-ordered treatment
18 pursuant to subsection A of this section.

19 2. There is reasonable cause to believe that the patient is an
20 incapacitated person as defined in section 14-5101 or is in need of
21 protection pursuant to section 14-5401, paragraph 2.

22 3. The patient does not have a guardian or conservator and the
23 welfare of the patient requires immediate action to protect the patient or
24 the ward's property.

25 4. The conditions prescribed pursuant to section 14-5310,
26 subsection B or section 14-5401.01, subsection B have been met.

27 I. The court may appoint as a temporary guardian or conservator
28 pursuant to subsection H of this section a suitable person or the public
29 fiduciary if there is no person qualified and willing to act in that
30 capacity. The court shall issue an order for an investigation as
31 prescribed pursuant to subsection G of this section and, unless the
32 patient is represented by independent counsel, the court shall appoint an
33 attorney to represent the patient in further proceedings regarding the
34 appointment of a guardian or conservator. The court shall schedule a
35 further hearing within fourteen days on the appropriate court calendar of
36 a court that has authority over guardianship or conservatorship matters
37 pursuant to this title to consider the continued need for an emergency
38 temporary guardian or conservator and the appropriateness of the temporary
39 guardian or conservator appointed, and shall order the appointed guardian
40 or conservator to give notice to persons entitled to notice pursuant to
41 section 14-5309, subsection A or section 14-5405, subsection A. The court
42 shall authorize certified letters of temporary emergency guardianship or
43 conservatorship to be issued on presentation of a copy of the court's
44 order. If a temporary emergency conservator other than the public
45 fiduciary is appointed pursuant to this subsection, the court shall order
46 that the use of the monies and property of the patient by the conservator

1 be restricted and not be sold, used, transferred or encumbered, except
2 that the court may authorize the conservator to use monies or property of
3 the patient specifically identified as needed to pay an expense to provide
4 for the care, treatment or welfare of the patient pending further hearing.

5 This subsection and subsection H of this section do not:

6 1. Prevent the evaluation [AGENCY] or [MENTAL HEALTH] treatment
7 agency from seeking guardianship and conservatorship in any other manner
8 allowed by law at any time during the period of court-ordered evaluation
9 and treatment.

10 2. Relieve the evaluation [AGENCY] or [MENTAL HEALTH] treatment
11 agency from its obligations concerning the suspected abuse of a vulnerable
12 adult pursuant to title 46, chapter 4.

13 J. If, on finding that a patient meets the criteria for
14 court-ordered treatment pursuant to subsection A of this section, the
15 court also learns that the patient has a guardian appointed under title
16 14, the court with notice may impose on the existing guardian additional
17 duties pursuant to section 14-5312.01. If the court imposes additional
18 duties on an existing guardian as prescribed in this subsection, the court
19 may determine that the patient needs to continue treatment under a court
20 order for treatment and may issue the order or determine that the
21 patient's needs can be adequately met by the guardian with the additional
22 duties pursuant to section 14-5312.01 and decline to issue the court order
23 for treatment. If at any time after the issuance of a court order for
24 treatment the court finds that the patient's needs can be adequately met
25 by the guardian with the additional duties pursuant to section 14-5312.01
26 and that a court order for treatment is no longer necessary to ensure
27 compliance with necessary treatment, the court may terminate the court
28 order for treatment.

29 K. The court shall file a report as part of the court record on its
30 findings of alternatives for treatment.

31 L. Treatment shall not include psychosurgery, lobotomy or any other
32 brain surgery without specific informed consent of the patient or the
33 patient's legal guardian and an order of the superior court in the county
34 in which the treatment is proposed, approving with specificity the use of
35 the treatment.

36 M. The medical director [OF A MENTAL HEALTH TREATMENT AGENCY] or
37 any person, agency or organization used by the medical director [OF A
38 MENTAL HEALTH TREATMENT AGENCY] to supervise the terms of an outpatient
39 treatment plan is not civilly liable for any acts committed by a patient
40 while on outpatient treatment if the medical director [OF THE MENTAL
41 HEALTH TREATMENT AGENCY], person, agency or organization has in good faith
42 followed the requirements of this section.

43 N. A peace officer who in good faith apprehends and transports a
44 patient to an inpatient treatment facility on the order of the medical
45 director of the outpatient treatment facility pursuant to subsection E,
46 paragraph ~~[6-]~~ [Z] of this section is not subject to civil liability.

1 O. If a person has been found, as a result of a mental disorder, to
2 constitute a danger to self or others or to have a persistent or acute
3 disability or a grave disability and the court enters an order for
4 treatment pursuant to subsection A of this section, the court shall
5 transmit the person's name, sex, date of birth, social security number, if
6 available, and date of the order for treatment to the supreme court. The
7 supreme court shall transmit the information to the department of public
8 safety to comply with the requirements of title 13, chapter 31 and title
9 32, chapter 26. The department of public safety shall transmit the
10 information to the national instant criminal background check system. The
11 superior court may access the information of a person who is ordered into
12 treatment to enforce or facilitate a treatment order.

13 P. On request, the clerk of the court shall provide certified
14 copies of the commitment order to a law enforcement or prosecuting agency
15 that is investigating or prosecuting a prohibited possessor as defined in
16 section 13-3101.

17 Q. If the court does not find a person to be in need of treatment
18 and a prosecutor filed a petition pursuant to section 13-4517, the
19 evaluation agency, within twenty-four hours, shall notify the prosecuting
20 agency of its finding. The court shall order the medical director [OF THE
21 EVALUATION AGENCY] to detain the person for an additional twenty-four
22 hours to allow the prosecuting agency to be notified. If the court has
23 retained jurisdiction pursuant to section 13-4517, subsection C, the court
24 may remand the person to the custody of the sheriff for further
25 disposition pursuant to section 13-4517, subsection A, paragraph 2 or 3.

26 R. After an order for treatment has been issued pursuant to this
27 section, the superior court in a county where a patient under a court
28 order for treatment is found or resides has concurrent jurisdiction with
29 the court in the county that issued the court order for treatment for the
30 purposes of enforcing the court order for treatment, ordering changes to
31 the treatment plan or amending the order to require the patient to undergo
32 further inpatient treatment. If the court in which proceedings are
33 commenced to enforce or administer the order for treatment is not the
34 court that originally entered the order for treatment, unless prevented by
35 an emergency, the court in which the proceedings are pending shall consult
36 with the court of original entry and determine whether to hold hearings
37 and enter orders to facilitate enforcement or administration of the court
38 order, whether to refer the case back to the court of original entry for
39 further proceedings or whether to transfer the entire case to the court of
40 original entry in that county for all further proceedings. The supreme
41 court may adopt rules to govern the procedures to be used in enforcing and
42 administering court orders for treatment in the various counties of this
43 state and the transfer of cases between counties involving court orders
44 for treatment.

45 S. Pursuant to the authority granted in subsection R of this
46 section, for the purpose of enforcing or facilitating treatment of a

1 patient under an active order for treatment, the supreme court shall adopt
2 a rule to establish a program to enable the judges of the superior court,
3 county attorneys, patients' attorneys, health care institutions as defined
4 in section 36-401 that provide services subject to the emergency medical
5 treatment and labor act (P.L. 99-272; 100 Stat. 164; 42 United States Code
6 section 1395dd), the regional behavioral health authority and behavioral
7 health service providers in any county to determine the existence of an
8 active court order for treatment and the history of court orders for
9 treatment entered for a patient by a superior court in any county in this
10 state. The program shall ensure that the information shared with other
11 persons or entities is necessary only for the purposes stated in this
12 subsection and shall require that the information shared be maintained as
13 confidential by the receiving person or entity.>>

14 Sec. 5. Section 36-543, Arizona Revised Statutes, is amended to
15 read:

16 36-543. Annual review; patients with a grave disability or a
17 persistent or acute disability; notice; court order
18 for continued treatment; rules; immunity

19 A. Within ninety days before the expiration of a court order for
20 treatment, the medical director of the mental health treatment agency
21 shall conduct an annual review of a patient who has been found to have a
22 grave disability or a persistent or acute disability and who is undergoing
23 court-ordered treatment to determine whether the continuation of
24 court-ordered treatment is appropriate and to assess the needs of the
25 patient for guardianship or conservatorship, or both. The annual review
26 shall consist of the mental health treatment and clinical records
27 contained in the patient's treatment file. The mental health treatment
28 agency shall keep a record of the annual review. If the medical director
29 OF THE MENTAL HEALTH TREATMENT AGENCY believes that a continuation of
30 court-ordered treatment is appropriate, the medical director of the mental
31 health treatment agency shall appoint one or more psychiatrists to carry
32 out a psychiatric examination of the patient. In any proceeding conducted
33 pursuant to this section, a patient has the right to have an analysis of
34 the patient's mental condition by an independent evaluation pursuant to
35 section 36-538.

36 B. Each examiner participating in the psychiatric examination of
37 the patient shall submit a report to the medical director of the mental
38 health treatment agency that includes the following:

39 1. The examiner's opinions as to whether the patient continues to
40 have a grave disability or a persistent or acute disability as the result
41 of a mental disorder and be in need of continued court-ordered
42 treatment. In evaluating the patient's need for continued court-ordered
43 treatment, the examiner must consider, along with all other evidence, the
44 patient's history before and during the current period of court-ordered
45 treatment, the patient's compliance with recommended treatment and any

1 other evidence relevant to the patient's ability and willingness to follow
2 recommended treatment with or without a court order.

3 2. A statement as to whether suitable alternatives to court-ordered
4 treatment are available.

5 3. A statement as to whether voluntary treatment would be
6 appropriate.

7 4. A review of the patient's status as to guardianship or
8 conservatorship, or both, the adequacy of existing protections of the
9 patient and the continued need for guardianship or conservatorship, or
10 both. If the examiner concludes that the patient's needs in these areas
11 are not being adequately met, the examiner's report shall recommend that
12 the court order an investigation into the patient's needs.

13 5. If the patient has an existing guardian who does not have the
14 mental health powers authorized pursuant to section 14-5312.01, a
15 recommendation as to whether the additional mental health powers
16 authorized by section 14-5312.01 should be imposed on the existing
17 guardian and whether the patient's needs can be adequately addressed by a
18 guardian with mental health powers without the need for a court order for
19 treatment or whether the court order for treatment should continue
20 regardless of the additional mental health powers imposed on the guardian.

21 6. The results of any physical examination conducted during the
22 period of court-ordered treatment if relevant to the psychiatric condition
23 of the patient.

24 C. After conducting the annual review as prescribed in this
25 section, if the medical director OF THE MENTAL HEALTH TREATMENT AGENCY
26 believes that continued court-ordered treatment is necessary or
27 appropriate, not later than thirty days before the expiration of the court
28 order for treatment, the medical director OF THE MENTAL HEALTH TREATMENT
29 AGENCY shall file with the court an application for continued
30 court-ordered treatment alleging the basis for the application and shall
31 file simultaneously with the application any psychiatric examination
32 conducted as part of the annual review. If the patient is under
33 guardianship, the medical director OF THE MENTAL HEALTH TREATMENT AGENCY
34 shall mail a copy of the application to the patient's guardian.

35 D. If an application for continued court-ordered treatment is
36 filed, all of the following apply:

37 1. If the patient does not have an attorney, the court shall
38 appoint an attorney to represent the patient.

39 2. Within ten days after appointment, an attorney appointed
40 pursuant to this subsection, to the extent possible, shall fulfill the
41 duties imposed pursuant to section 36-537, review the ~~medical director's~~
42 report OF THE MEDICAL DIRECTOR OF THE MENTAL HEALTH TREATMENT AGENCY and
43 the patient's medical records, interview any physician who prepared a
44 report on the annual review and file a response requesting a hearing or
45 submitting the matter to the court for a ruling based on the record
46 without a hearing.

1 3. If a hearing is not requested, the court shall rule on the
2 application or set the matter for hearing. If a hearing is requested, the
3 hearing shall be held within three weeks after the request for hearing is
4 filed. The hearing may be continued for good cause on motion of a party
5 or on the court's own motion, and the expiration of the current court
6 order for treatment may be extended until a ruling by the court on an
7 application filed pursuant to this subsection.

8 4. The patient's attorney must be present at all hearings and may
9 subpoena and cross-examine witnesses and present evidence. The patient
10 has the right to attend all hearings, but may choose not to attend a
11 hearing. The patient's attorney may waive the patient's presence after
12 speaking with the patient and confirming that the patient understands the
13 right to be present and does not desire to attend. If the patient is
14 unable to be present at the hearing for medical or psychiatric reasons and
15 the hearing cannot be conducted where the patient is being treated or
16 confined, or the patient cannot appear by another reasonably feasible
17 means, the court shall require clear and convincing evidence that the
18 patient is unable to be present at the hearing and on such a finding may
19 proceed with the hearing in the patient's absence.

20 5. The evidence presented by the applicant includes the testimony
21 of one or more witnesses acquainted with the patient during the period of
22 court-ordered treatment, which may be satisfied by a statement agreed on
23 by the parties, and the testimony of any physician who performed an annual
24 review of the patient, which may be satisfied by stipulating to the
25 admission of the examining physicians' written report prepared pursuant to
26 subsection B of this section. The court may waive the need for the
27 applicant to present the testimony of witnesses acquainted with the
28 patient as required by this subsection, if it finds that the need for a
29 continued court order for treatment has been established by clear and
30 convincing evidence from the other testimony and evidence presented at the
31 hearing. THE COURT, FOR GOOD CAUSE SHOWN, MAY ALLOW THE PATIENT, THE
32 PATIENT'S ATTORNEY OR ANY WITNESS TO APPEAR AND PRESENT TESTIMONY IN OPEN
33 COURT THROUGH THE USE OF TELEPHONE, VIDEOCONFERENCING OR OTHER AUDIOVISUAL
34 TECHNOLOGY.

35 6. At a hearing held pursuant to this subsection, the court, with
36 notice, may impose on an existing guardian additional powers pursuant to
37 section 14-5312.01. If the court finds that the patient's needs can be
38 adequately met by an existing guardian with the additional powers pursuant
39 to section 14-5312.01 and that a court order for treatment is not
40 necessary to ensure compliance with necessary treatment, the court may
41 terminate the court order for treatment or decline to issue an order
42 continuing court-ordered treatment. The court may also order an
43 investigation into the need for guardianship or conservatorship, or both,
44 and may appoint a suitable person or agency to conduct the investigation.
45 The appointee may include a court-appointed guardian ad litem, a
46 court-appointed investigator pursuant to section 14-5308 or the public

1 fiduciary if there is no person willing and qualified to act in that
2 capacity. The court shall give notice of the appointment to the appointee
3 within three days after the appointment. The appointee shall submit the
4 report of the investigation to the court within twenty-one days. The
5 report shall include recommendations as to who should be guardian or
6 conservator, or both, and the findings and reasons for the recommendation.
7 If the investigation and report so indicate, the court may authorize an
8 appropriate person to file a petition for appointment of a guardian or
9 conservator for the patient.

10 E. If a hearing is held pursuant to subsection D of this section,
11 the party seeking the renewal of the court order must prove all of the
12 following by clear and convincing evidence:

13 1. The patient continues to have a mental disorder and, as a result
14 of that disorder, has either a persistent or acute disability or a grave
15 disability.

16 2. The patient is in need of continued court-ordered treatment.

17 3. The patient is either unwilling or unable to accept treatment
18 voluntarily.

19 F. After a hearing held pursuant to subsection D of this section,
20 the court may order the patient to be released from court-ordered
21 treatment or to undergo continued court-ordered treatment for a period not
22 to exceed the time periods prescribed in section 36-540, subsection D.

23 G. The director shall create and operate a program to ensure that
24 the examination and review of persons with grave disabilities or
25 persistent or acute disabilities under court order are carried out in an
26 effective and timely manner. The director shall adopt rules needed to
27 operate this program.

28 H. The medical director of the mental health treatment agency is
29 not civilly liable for any acts committed by the released patient if the
30 medical director OF THE MENTAL HEALTH TREATMENT AGENCY has in good faith
31 complied with the requirements of this article.

32 <<Sec. 6. Section 36-550.09, Arizona Revised Statutes, is amended
33 to read:

34 36-550.09. Secure behavioral health residential facilities;
35 court determination; findings

36 A. If a court finds that a patient meets the criteria for
37 court-ordered treatment pursuant to section 36-540, subsection A, the
38 court may approve the patient's placement in a secure behavioral health
39 residential facility that is licensed by the department pursuant to
40 section 36-425.06 and that is willing to accept the patient if the patient
41 has been determined to be seriously mentally ill and the court finds that
42 the patient is chronically resistant to treatment as set forth in this
43 section. Placement in a [LICENSED] secure behavioral health residential
44 facility for treatment is not a period of inpatient treatment for the
45 purposes of section 36-540, subsection F.

1 B. A court may determine that a person is chronically resistant to
2 treatment if the court finds that, within twenty-four months before the
3 issuance of a court order pursuant to this section, excluding any time
4 during this period that the person was hospitalized or incarcerated, the
5 person demonstrated a persistent or recurrent unwillingness or inability
6 to participate in or adhere to treatment for a mental disorder despite
7 having treatment offered, prescribed, recommended or ordered to improve
8 the person's condition or to prevent a relapse or harmful deterioration of
9 the person's condition. The court's finding shall be based on evidence
10 that establishes all of the following by clear and convincing evidence:

11 1. The person received treatment in the preceding twenty-four
12 months in other less-restrictive settings, including unsecured residential
13 treatment settings with on-site twenty-four-hour supportive treatment and
14 supervision by staff with behavioral health training, and the treatment
15 was unsuccessful or is not likely to be successful due to the person's
16 expressed or demonstrated unwillingness to cooperate with treatment in
17 other less-restrictive or unsecured residential treatment settings.

18 2. The person's nonadherence to or nonparticipation in treatment
19 over the preceding twenty-four months resulted in one or more of the
20 following:

21 (a) Serious harm to self.

22 (b) Serious harm or threats of serious harm to others.

23 (c) Recurrent periods of homelessness resulting from the mental
24 disorder.

25 (d) Recurrent serious medical problems due to poor self-care or
26 failure to follow medical treatment recommendations.

27 (e) Recurrent arrests due to behavior resulting from the mental
28 disorder.

29 3. Any other evidence relevant to the person's willingness or
30 ability to participate in and adhere to treatment or the person's need for
31 treatment in a licensed secure residential setting to ensure the person's
32 compliance with court-ordered treatment.

33 C. A person's placement in a licensed secure behavioral health
34 residential facility for treatment shall be part of the written treatment
35 plan [THAT CONFORMS TO THE REQUIREMENTS OF SECTION 36-540.01, SUBSECTION B
36 AND THAT IS] presented to [THE COURT BY A MENTAL HEALTH TREATMENT AGENCY]
37 and approved by the court [as required by section 36-540, subsection C,
38 paragraph 2]. The court shall confirm in the order that the person's
39 placement in a licensed secure behavioral health residential facility is
40 the least restrictive environment to ensure the person's compliance with
41 the treatment plan. [THE MENTAL HEALTH TREATMENT AGENCY THAT WILL
42 SUPERVISE THE TREATMENT PURSUANT TO SECTION 36-540, SUBSECTION E AND THE
43 LICENSED SECURE BEHAVIORAL HEALTH RESIDENTIAL FACILITY SHALL COORDINATE
44 THE CARE OF THE PATIENT PURSUANT TO RULES ADOPTED BY THE DEPARTMENT OR AS
45 REQUIRED BY THE ADMINISTRATION.

1 D. A PERSON WHO IS PLACED IN A LICENSED SECURE BEHAVIORAL HEALTH
2 RESIDENTIAL FACILITY MAY APPEAR AND PRESENT TESTIMONY IN OPEN COURT
3 THROUGH THE USE OF TELEPHONE, VIDEOCONFERENCING OR OTHER AUDIOVISUAL
4 TECHNOLOGY PURSUANT TO SECTION 36-505, SUBSECTION B.]>>
5 <<Sec. 7. Retroactivity
6 [Sections 36-501, 36-540 and 36-550.09, Arizona Revised Statutes, as
7 amended by this act, apply retroactively to from and after April 31,
8 2025.]>>

9 Enroll and engross to conform
10 Amend title to conform

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